MEDICAL CASE PRESENTATION

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DEMOGRAPHIC DETAILS

- A 54 years aged female hailing from Villianur who is a farmer by occupation, of lower-middle socio-economic status, came with chief complaints of :-

- Fever for past 2 months
- Cough with expectoration for 1 month
- Shortness of breath for 1 month

HISTORY OF PRESENTING ILLNESS

- Patient was apparently asymptomatic 2 months ago following which she developed fever which was continuous, low grade fever associated with evening rise of temperature, with no chills or rigors and patient used to take single tablet of paracetamol for getting relief.
- Patient also c/o cough for past 1 month, present throughout the day, associated with sputum production which is mucoid in nature, scanty in amount, not foul-smelling, not blood stained, with no postural or diurnal variation and no aggravating or relieving factors. Not associated with wheeze.

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- Patient also complaints of breathlessness for past 1 month, insidious in onset gradually progressive, initially patient had breathlessness on walking upstairs but now has breathlessness even while walking a certain distance (Grade 1 to Grade 3 MMRC). It aggravates on lying on the left side and on coughing but partially relieved on lying on right side.
- Patient had c/o chest pain, acute onset, in left axilla, stabbing in nature, aggravated on inspiration and coughing relieved on lying on left side and now there is no pain for past 10 days but only tightness over the left chest.
- No h/o paroxysmal nocturnal dyspnea or orthopnea
- No h/o halitosis
- There is also h/o loss of weight of about 7 kg in the past 2 months and reduced appetite
- No h/o swelling in the legs, syncope, puffiness of face or reduced urine output
- No h/o skin rashes or joint pain
- No h/o change in voice or difficulty in swallowing .
- No h/o any lump in the breast, neck swellings

PAST HISTORY

- No h/o similar illness in the past
- N/K/C/O Diabetes, Hypertension, Bronchial Asthma, Collagen vascular diseases, Malignancies
- No h/o contact with a patient with TB
- No h/o any previous surgeries

PERSONAL HISTORY

- Patient consumes a mixed diet
- She has a normal sleep wake cycle
- She has normal bowel and bladder movements
- There is no h/o any alcohol intake or paan chewing or substance abuse
- She works as a 100 days worker in the paddy fields

FAMILY HISTORY

- No h/o similar illnesses in the family
- Her husband is also a farmer and for past 30 years smokes atleast 5 bidis/day
- H/o indoor air pollution and overcrowding is present
- No h/o TB or any malignancies in the family

SUMMARY

- A 54 years aged female came with c/o fever for 2 months, continuous low grade fever with evening rise of temperature and no chills and rigors. She c/o cough with expectoration with scanty sputum not blood stained, no postural or diurnal variation and cough is present throughout the day and not a/w wheeze
- She also c/o progressive breathlessness aggravates on coughing and on lying right side and partially relieved on lying on left side
- She has unintentional significant weight loss of 7 kg in past 2 months and reduced appetite
- All the points indicate a chronic respiratory disease, probably pleural in origin and I would like to examine the respiratory system

PROVISIONAL DIAGNOSIS

- Pleural Effusion
- Malignancy (Bronchogenic Carcinoma, Lymphoma)
- Interstitial Lung Disease
- Hypersensitivity Pneumonitis
- Idiopathic Pulmonary Fibrosis
- Sarcoidosis
- Chronic Mediastinitis
- Subacute Bacterial Endocarditis

GENERAL PHYSICAL EXAMINATION

- Patient's consent is obtained and she is in a well-lit room.
- Patient is conscious and oriented to time, place and person
- She is moderately built and poorly nourished
- BMI - 18 kg/sq.metres
- She prefers to lie in right lateral decubitus
- There is no pallor/icterus/cyanosis/clubbing/lymphadenopathy/pedal edema
- No external markers of TB, HIV, Pulmonary malignancy

- **VITALS** –

Pulse – 110 beats per minute, normal in volume, no specific character, measured in right radial artery

RR – 26 cycles per minute, thoraco-abdominal in character, accessory muscles are not active

BP – 100/70 mmHg, taken in the right upper arm in supine position

Temperature – 100.8 deg.F

Saturation - 87%

SYSTEMIC EXAMINATION

- **INSPECTION** –
- **Upper Respiratory Tract** – nasal septum in midline, no nasal polyps, sinus is normal, no halitosis, oral cavity is normal and tonsils are normal

- **Lower Respiratory Tract** – The chest is normal in shape

Trachea is deviated to the right side

There is restricted chest wall movements on the left side

Fullness of left side intercostal spaces are seen

Apical impulse is not seen

No drooping of shoulders, supra & infra-clavicular indrawing or fullness

No crowding of ribs

No scars/sinuses/dilated veins veins over chest

Spine is normal and no prominence of medial border of scapula

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- **PALPATION** :- Trachea is deviated to the right side, no intercostal tenderness

Apical impulse cannot be felt

Movements of the chest are decreased on the left side

MEASUREMENTS :-

AP diameter - 22 cm

Transverse diameter - 32 cm

RATIO – 5.5 : 8

Chest circumference – at expiration – 82 cm

at inspiration – 85 cm

Total chest expansion is 3 cm

Hemithoracic expansion – RIGHT – 40 cm ; on inspiration – 43 cm

LEFT - 41 cm ; on inspiration – 42 cm

LEFT hemithorax < RIGHT hemithorax

Spino-scapular distance - normal

Spino-acromial distance - Normal

Contd…

VOCAL FREMITUS	RIGHT	LEFT
SUPRACLAVICULAR	NORMAL	NORMAL
INFRACLAVICULAR	NORMAL	NORMAL
MAMMARY	NORMAL	ABSENT
AXILLARY	NORMAL	ABSENT
INFRA-AXILLARY	NORMAL	ABSENT
SUPRASCAPULAR	NORMAL	ABSENT
INTERSCAPULAR	NORMAL	ABSENT
INFRASCAPULAR	NORMAL	ABSENT

PERCUSSION

SITE OF PERCUSSION	RIGHT	LEFT
SUPRACLAVICULAR	NORMAL	NORMAL
INFRACLAVICULAR	NORMAL	NORMAL
MAMMARY	NORMAL	STONY DULL
AXILLARY	NORMAL	STONY DULL
INFRA-AXILLARY	NORMAL	STONY DULL
SUPRASCAPULAR	NORMAL	STONY DULL
INTERSCAPULAR	NORMAL	STONY DULL
INFRASCAPULAR	NORMAL	STONY DULL

- Tidal Percussion – no dullness
- Liver dullness - rt. 5th ICS, Liver span - 14 cm
- Traube’s Space – dull note
- No shifting dullness

AUSCULTATION

SITES OF AUSCULTATION	VESICULAR BREATH SOUNDS		ADDED SOUNDS	
	RIGHT	LEFT	RIGHT	LEFT
SUPRACLAVICULAR	NORMAL	NORMAL	NO	NO
INFRACLAVICULAR	NORMAL	NORMAL	NO	NO
MAMMARY	NORMAL	ABSENT	NO	NO
AXILLARY	NORMAL	REDUCED	NO	NO
INFRA-AXILLARY	NORMAL	ABSENT	NO	NO
SUPRASCAPULAR	NORMAL	REDUCED	NO	NO
INTERSCAPULAR	NORMAL	ABSENT	NO	NO
INFRASCAPULAR	NORMAL	ABSENT	NO	NO

	VOICE SOUNDS	
SITES	RIGHT	LEFT
SUPRACLAVICULAR	NO	NO
INFRACLAVICULAR	NO	NO
MAMMARY	NO	NO
AXILLARY	NO	NO
INFRA-AXILLARY	NO	NO
SUPRASCAPULAR	NO	NO
INTERSCAPULAR	NO	NO
INFRASCAPULAR	NO	NO

Contd…

VOCAL RESONANCE	RIGHT	LEFT
SUPRACLAVICULAR	NORMAL	NORMAL
INFRACLAVICULAR	NORMAL	NORMAL
MAMMARY	NORMAL	ABSENT
AXILLARY	NORMAL	ABSENT
INFRA-AXILLARY	NORMAL	ABSENT
SUPRASCAPULAR	NORMAL	ABSENT
INTERSCAPULAR	NORMAL	ABSENT
INFRASCAPULAR	NORMAL	ABSENT

OTHER SYSTEMS

- CVS – S1 & S2 not clearly heard, apex beat was not felt, no murmurs were heard
- GIT – soft and non tender abdomen, no organomegaly
- CNS – no focal neurological deficits

SUMMARY

- A 54 years aged female came with c/o fever for 2 months, continuous low grade fever with evening rise of temperature and no chills and rigors. She c/o cough with expectoration with scanty sputum not blood stained, no postural or diurnal variation and cough is present throughout the day and not a/w wheeze
- She also c/o progressive breathlessness aggravates on coughing and on lying right side and partially relieved on lying on left side
- She has unintentional significant weight loss of 7 kg in past 2 months and reduced appetite
- On examination, she is moderately built and nourished and has a respiratory rate of 26 cycles/minute. On inspection, there is intercostal fullness, restricted left side chest movements, trachea is deviated to right side and apical impulse cannot be made out. On palpation, inspectory findings were confirmed and left hemithoracic expansion is lesser than the right, vocal fremitus was absent on left side except on supra and infra-clavicular spaces
- On percussion, stony dull note was elicited except on supra and infra-clavicular spaces and Traube's Space was dull
- On auscultation, there is absent of breath sounds and no added sounds are heard and vocal resonance is absent except on supra and infra-clavicular spaces.

DIAGNOSIS

- A case of left-sided massive pleural effusion probably of tuberculous/malignant etiology not in respiratory failure or cor pulmonale without any complications

CHEST X-RAY PA VIEW

